2. Stigma, Shame and Silence

Kalpana Jain

Kalpana Jain, a renowned figure in health journalism, has written extensively on public health issues such as tuberculosis, malaria and recent outbreaks of fevers like dengue and chikungunya. She first received national attention as the health editor of *The Times of India*, specialising in health and population issues and then moved on to become a Kaiser International Health Journalism Fellow. Her greatest contribution to health journalism is perhaps her extensive coverage of the AIDS epidemic which she did with meticulous care, travelling to various affected parts of the country and gathering information on and from several HIV positive people. She succeeded in making people aware of this frightening new health hazard and sensitising her readers towards the issue.

As the editor of *Health* e-Letter, Jain researches and publishes on a variety of health issues. She is also an advocate of people's rights. 'Stigma, Shame and Silence' is an extract from her book *Positive Lives: The Story of Ashok and Others with HIV* (2003). In this book she throws light on the social stigma and shame faced by people diagnosed as HIV-positive. The book also describes amazing stories of hundreds of men and women who have conquered their shame and fear thereby becoming sources of inspiration for others infected with the virus. In this extract, Kalpana Jain exposes people's prejudices towards HIV-positive people and highlights the ambivalent attitude of even the medical establishment. The extract is an indictment of the medical establishment and the armed forces, highlighting their insensitivity in dealing with the afflicted populace. Through the use of a personal narrative, the extract points out how human

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rights are subtly violated in the aversion and indifference with which the afflicted are treated.

As a child, Ashok used to watch with glee his paper boats bobbing in the rainwater and then suddenly getting washed away. There was no sense of loss when this happened. All he had to do was to make another boat. Today, standing on the ship's deck, he was reminded of his childhood days; he wondered whether like those paper boats his life, too, would flow away into oblivion. The thought left him desolate and helpless, as never before. Overcome by depression, he began brooding: there was no one who really cared for him, no one who would even feel his loss when he was gone. Just knowing about his HIV status had given even the most inept guys amongst his shipmates,'a feeling of enormous power, a sense of unreasonable superiority, which led them to behave as they liked with him. The more he thought about it, the angrier he got. 'It is truly an unfair world,' he said to himself. The moment people around you felt you were weaker than them, they went all out to get you. And because he had got HIV, even his friends had arrogated to themselves the right to play with his feelings, to rough up his emotions. Perhaps, another person in his place would not have been able to take these constant humiliations. For a long time, Ashok intently kept observing the waves as they lashed against the ship. Unpleasant thoughts flashed through his mind as he tried to figure out how he should go about ordering his new life. Soon he realized this was all a matter of how he channelled his energies. They should certainly not be frittered away in anger, he decided. The best way he could deal with the rejection he felt, was to learn to ignore it. Also, if he kept out of everyone's way, he wouldn't have to bear the torture of indifference. To survive, he would have to make a habit of pretending not to hear those rude remarks, as well as try not to notice people's contemptuous attitude towards him. He couldn't afford to get into any arguments or fights. Only he would be the loser. So, he counseled himself. Above all, he did not want to give others more of an opportunity to express their superiority.

In this way, he learnt to protect himself early on, starting with the Command Hospital, Pune, where he was sent for testing.

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The doctor there had been brutal.

He had delivered the news to him like a death sentence.

No mincing of words.

No trappings of hope.

He had looked at him and said, 'You know, you are positive.' Just five words, conveyed with surgical precision.

It requires great strength of spirit to be able to bear such a sentence – a death sentence that is without appeal. It is worse than being a criminal who is to be hanged for violating the norms of civil society.

Ashok could feel the doctor point the finger at him; he was jeering at him, his eyes mocking him. He seemed to say, you deserve this for what you have done. There can be no sympathy for you. You are condemned to die for your deeds.

Had it been another terminal illness, things would have been so different. He could have reached out to his family. His mother, his sisters, would consult the best doctors and extend their support. In this, he was alone. This was his shame, and he had to hide it. Battling thoughts of early death, he had to continue living, to earn enough for himself and to send something home.

Ashok looked at the doctor. It seemed he was waiting for him to break down, sob uncontrollably and weep his heart out as he admitted his guilt. At this point the doctor would get up, put an arm around him, to offer him solace. Wearing that look of moral superiority Ashok detested, he'd add that atonement for his sins was possible if he did not commit them anymore.

Moral superiority? Would he be judged only by his HIV status henceforth? Looking up at the doctor again, he felt his entire body imploding with laughter. Somehow, he managed to control himself. The doctor was watching him. At least for the sake of form, Ashok scolded himself, he should appear to be composed.

The doctor believed the message had not registered. He persisted: 'You are suffering from AIDS. Do you understand? Your life is short. Your immunity will go down. You will get lots of diseases. Eventually, you'll die.' He looked up, quite confident that Ashok must be terrified by now and would beg for mercy. There were no

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another thought. Little did he know that would be the last of his normal working days. The next day he was in hospital. Initially, no one told him what they were suspecting. He was there for four weeks and God knows how many blood tests he had to undergo. The male nursing assistant would just come, roll up his sleeve, look for his vein and plunge in the needle. So used to this had he grown that just thinking about it set off a throbbing sensation in his arm. He always seemed so stern and the starched uniform only emphasized his stiffness. How could Ashok think of asking him anything about what was happening? The entire set-up was so intimidating.

He recalled how he had checked with all the sailors when he saw the name of a blood test written on the slip attached to his bed: Elisa for HIV. Of course, no one thought anything of it at that time. But he did remember that one of the sailors remarked, 'This is the AIDS test,' and his comment had drawn a puzzled look from the rest. 'AIDS, why AIDS?' he sat there, feeling a little stunned. 'What has that got to do with me? I don't have any STDs.'

He was then told there was some problem with his blood; therefore, he had to be sent to the Command Hospital, Pune, to get a Western Blot test done. There were three others who had come from the Army and Navy units from various parts of the country. 'We were told at that time that all tests, like the TLC, DLC, and chest X-Ray, would be done. Samples were collected and everyone was taken to National Institute of Virology for another test. We had to wait four to five days for the final results.' In those five days, Ashok started preparing himself for the worst. 'Even if the verdict is AIDS, so what?' he would repeat to himself several times a day. Still, he was so confused: not that he had not heard about this virus, it's just that no one made him realize that it could happen to him or to anybody else like him. He had believed it happened to only those women – the prostitutes – and men who visit them. Maybe, he thought, maybe he did not have HIV. After all, he had healthy girlfriends. How could he have contracted the virus?

Those five days were also a preparation for the discrimination that lay ahead. He was kept in an isolation ward, meant just for AIDS patients. There were five others with him. No one would come and visit them. All that they did was play cards to amuse themselves and

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pass time. The television was kept in a common lounge. Several patients would come and watch television there. But all those in the isolation ward preferred to keep a distance.

What if someone checked them? What if someone was rude to them? They did not venture to cross the boundaries set for them by the hospital. They did not even attempt going through a general passage. If they had to go out, they would take a longer route. Most of those in the isolation ward were suffering from depression. They were worried about their families. Ashok too had similar worries. But he would always tell them, 'There is no point in opening the umbrella before it starts raining.'

Ashok's mind went back to the doctor's advice, 'Don't get married. And never have sex again.' 'How could the doctor think such advice would work?' he wondered. People living with HIV needed to be educated on how to have safe sex. They had to be informed about the need to protect others. People don't stop having sex after they have tested HIV positive, he thought, drifting off to sleep.

Ashok was given a 'No Abnormalities Detected' (NAD) certificate by the psychiatrist and discharged. On the way back, he was rehearsing how he would tell his friends about it. Little did he know that the hospital had already sent a letter to his commanding officer, who had then summoned a meeting of the division and announced. 'One sailor tested positive.' No guesses were required.

Everyone knew.

Welcome aboard: to a new world of stigma and shame.

Glossary

stigma: mark of shame or dishonour
glee: great delight
bob: to move up or down quickly
oblivion: a state of being completely disregarded and forgotten
HIV: human immunodeficiency virus, the virus that causes the acquired immune deficiency syndrome or AIDS
inept: incompetent; ineffective
arrogate: to appropriate or claim without any right
fritter: to waste

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mince words: to speak of an unpleasant thing without using plain, direct words
jeer: to mock
atone for: to make amends for wrong-doing
implode: to explode inwards
unhinge: to disrupt the balance of (used in reference to the mind)
herpes zoster: a viral disease characterised by painful skin rashes or blisters
blisters: thin watery swellings under the skin, often caused by continuous and forceful rubbing or burning
intimidate: to frighten
STD: (here) sexually transmitted disease
lounge: a comfortable room for sitting in

Comprehension

1. Answer the following in not more than two pages each.

a. Discuss Kalpana Jain's treatment of a serious social problem in this essay.
b. With reference to this essay, do you feel that there is a human rights issue in the treatment meted out to HIV-positive people? Give reasons for your answer.
c. What are the possible solutions that you can suggest to counter this situation?

2. Answer the following in not more than a page each.

a. How do you relate the significance of the title to Ashok's feelings and reactions on being told about his having AIDS.
b. The trauma of facing colleagues and society after being informed of one's illness such as AIDS can be extremely limiting. Explain this in the light of Ashok's situation.
c. Contracting AIDS is a signal for ostracisation. How does Ashok's naval unit take steps in this regard?

3. Answer the following in about 100 words each.

a. What is the attitude of the doctor here to HIV-positive patients?

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